

Part 1: Learning Objectives

By the end of Part 1 of this module students will be able to:

1. Describe and evaluate societal attitudes towards adolescent sexuality and sexual behaviour.
2. Identify the key developmental tasks related to sexuality for adolescents.
3. Describe patterns of sexual behaviour during the adolescent years.
4. Identify and distinguish between the various influences on adolescent sexuality.

Topic Overview

Introduction: The biological and social context of adolescent sexuality

It is perhaps stating the obvious to say that adolescence is a challenging part of our lives. One of the things that make it so challenging is that during adolescence nearly every aspect of our lives changes in profound ways. The physical, intellectual, emotional, and social domains of life are in state of rapid change beginning at puberty until we enter young adulthood in our early twenties. Of course, one of the areas where these changes are most prominent is sexuality.

Sexual self-awareness may begin prior to adolescence and continues at an accelerated pace during adolescence and there is no doubt that, for most people, adolescence is a 'critical period' in the upsurge of sexual drives, the development of sexual values, and the initiation of sexual behaviours. The onset of puberty, the power of peer group expectations, and the communication of mixed messages about sex from adults can make dealing with sexuality difficult but for many, and hopefully most adolescents, the development of sexuality can be a positive and life enriching experience.

Key Point

Adolescence is a time of profound change in all aspects of life including sexuality.

As we saw in Module/Week 6, many of the building blocks of adult sexuality are built in childhood, but it is with the onset of puberty that we swiftly start to become sexual beings in the adult sense. In looking at the biological and social context of adolescent sexuality, it is important to note that beginning with puberty, youth are biologically capable of reproduction, have sexual feelings and desires, but, in Western society at least, they are about a decade away from assuming full adult status.

We should keep in mind that until the twentieth century, in many respects, there was no such thing as adolescence. Soon after puberty, if not before, most people were out of school and often were married before their twentieth birthday. Obviously, this is not what happens for most Canadian youth today. According to sociologist Eleanor Maticka-Tyndale (2001),

What is new is the length of adolescence - that time between childhood and entry into roles are markers of adulthood (such as husband, wife, or parallel status; employee in a permanent job; rentee or mortgagee). Adolescence is a time when one is biologically mature but kept socially (and cognitively) immature, i.e., not ready to be an adult (p.2).

Furthermore,

Lets be clear about this - these are social developments that are not tied to biology. Though young men and women enter puberty before they enter their teens, the average age of marriage is approaching 30. This means that by the time they marry young people have been biologically capable of reproduction for more than half their lives (Maticka-Tyndale, 2001, p. 2).

This gap between the biological readiness for sexual activity and the age at which we gain adult status has significant implications for how our society addresses adolescent sexuality. Although most teenagers are biologically ready to have sex, it is often said that teenagers are "not mature enough" to engage in sexual activity.

Key Point

In contemporary Western culture there is a significant gap between when young people become biologically capable of reproduction and the age at which most adults approve of young people having sex.

The Developmental Tasks of Adolescent Sexuality

For this course, we are looking at each phase of the life cycle in terms of specific developmental tasks related to sexuality. During adolescence, these developmental tasks are numerous and often difficult to accomplish. In one respect, the sexuality related developmental tasks of adolescence are confusing for the simple reason that adolescents live in a culture where they are biologically ready for sex, are constantly exposed to sexual imagery in the media, but at the same time they are often discouraged from expressing their sexuality by their parents, schools, and religious institutions. It is against the backdrop of this contradictory social context that we consider five major developmental tasks of adolescent sexuality.

Key Point

The developmental tasks of adolescent sexuality can be divided into five major categories:

1. Becoming comfortable with a rapidly changing body;
2. Gaining independence from parents/family;
3. Developing self-acceptance as a sexual being;
4. Exploring romantic/sexual relationships;
5. Protecting sexual health;

- 1. Becoming comfortable with a rapidly changing body:** On average girls will begin puberty at age 10 or 11 and boys will begin at age 11 or 12. As anyone who has been through puberty knows, it is a time when both the mind and body are in a state of constant, often disconcerting change. (For an outline of the precise physical changes involved in puberty see Chapter 12 of your textbook). Physically, rapid increases in growth, development of secondary sex characteristics (e.g., pubic hair, facial hair, increased breast and genital growth), development of acne and body odour, menstruation, and other physical changes leaves many adolescents feeling awkward, insecure, and uncomfortable about their bodies. At one point or another, nearly all adolescents will wonder if their bodies are "normal" or will become convinced that they are unattractive. Media images depicting very narrow and unrealistic representations of the ideal body as well as peer group expectations can be very discouraging for adolescents struggling to gain confidence in themselves. On a general level, it is important for a person's overall well-being to become comfortable with their body. With respect to sexuality specifically, people who are comfortable with and accepting of their bodies are more likely to be comfortable and confident in their sexual relationships throughout their lives.
- 2. Gaining independence from parents/family:** Many theorists studying human development suggest that adolescence is a time when young people are seeking to establish a sense of their own identity as an individual. Put simply, adolescents are asking themselves "Who am I?" As part of the process of developing their own sense of self, adolescents will typically attempt to distance themselves from their parents and more closely align with their friends and peer group. Very often this move towards a sense of independence involves a rejection of parental values and norms including those related to sexuality. For example, some adolescents will reject their parent's wishes with respect to teen sexual behaviour, particularly if parental opinions conflict with teen peer group norms, as a way of asserting their independence and fulfilling a need to feel like an adult. Part of the general process of asking "Who am I?" is asking the question "Who am I as a sexual person?"
- 3. Developing Self-Acceptance as a Sexual Being:** It's especially important that adolescents are able to integrate their sexual feelings, needs, and desires into a positive self identity, which includes, as one aspect, a sexual self. Put more simply, it is important for an adolescent to fully and consciously accept that they are a sexual being with sexual desires, needs, and feelings. We might assume, given the relatively open and permissive approach to sexuality in contemporary Western culture, that acceptance of the self as a sexual being is something that happens as a matter of course with all adolescents. However, some adolescents do have difficulty accepting themselves as sexual beings. This may particularly be the case for LGBTQI2S+ adolescents who have been exposed to homophobic attitudes. Fortunately, this is beginning to change as society becomes more accepting of diverse sexual identities, sexual minority youth navigate the developmental tasks of adolescent sexuality as well as their peers. Although Western culture has moved forward in recognizing and affirming female sexuality, to some extent, a sexual double standard between male and female adolescents still exists. In other words, our society is relatively more accepting of the sexual behaviour of teenage boys but looks more negatively upon teenage girls who are sexually active. Developing self-acceptance as a sexual being is important not only in terms of an adolescent's ability to form healthy current and future relationships. It is also important because people who do not acknowledge or accept themselves as sexual beings are less likely to take measures to protect their sexual health. For example, young people who do not acknowledge or accept themselves as sexual beings are less likely to plan to have sex or recognize that they might have sex. As a result, they are much less likely to practice safer sex if they do engage in sexual activity. This probably explains a significant proportion of unintended pregnancies and cases of STIs.
- 4. Exploring Romantic/sexual Relationships:** It is during adolescence that most young people will have their first romantic and/or sexual relationship. Like most other aspects of teenage social life, romantic relationships typically cause a great deal of anxiety for adolescents. Many will have their first experience with falling in love and will be faced with the emotional turmoil of beginning and ending relationships. Adolescents are faced with the developmental task of learning, often with little or no experience, how to be in a mutually beneficial relationship and the experience they have with this process can have a significant influence on how they approach relationships throughout their lives.
- 5. Protecting Sexual Health:** As you will see in your required reading for this module, the vast majority of Canadians become sexually active during adolescence and many teens are at high risk for HIV/STIs and unintended pregnancy. Thus, learning the information and skills to consistently engage in behaviours that protect sexual health is a major developmental task of adolescent sexuality. Later we will explore how educational programs can effectively equip adolescents with the information, motivation, and behavioural skills to protect their sexual health.

Patterns of Sexual Behaviour During the Adolescent Years

Key Point

The majority of Canadian young people will become sexually active by the time they reach age 20.

The textbook documents various forms of sexual behaviour that occur in adolescence (i.e., masturbation, oral sex, intercourse). Approximately two thirds of Canadians will experience oral sex and/or sexual intercourse before age 20. The most recent Statistics Canada data on sexual activity among young people indicates that the percentages of Canadian youth aged 15 to 24 who have experienced intercourse has not been increasing in recent years and may even be declining (Rotermann & McKay, 2020; SIECCAN, 2020). When reading about adolescent sexual behaviour, keep the following three Key Points in mind.

Key Point

Adolescent sexual behaviour is characterized by serial monogamy.

1) It is important to note that although most youth become sexually active at some point during adolescence, their sexual behaviour is often sporadic. That is, many, if not most, adolescents do not have sex on a regular or consistent basis. The literature on adolescent sexuality often designates a teenager who has had sex once as being "sexually active" even if that one time occurred several years in the past. However, it is not uncommon for a teenager, particularly a younger teen, to have sex only a few times over a number of years and to have a small number of sexual partners. Part of this is due to the fact that adolescent romantic/sexual relationships are often relatively short in duration compared to adult relationships.

Nevertheless, it is important to recognize that there is a subset of adolescents who are highly sexually active both in terms of frequency and number of sexual partners.

Key Point

Adolescent sexual behaviour is often sporadic and adolescent romantic/sexual relationships tend to be relatively short in duration.

2) It is important to consider the concept of serial monogamy in order to understand patterns of sexual behaviour among adolescents and young adults. Most (but certainly not all) sexually active people are monogamous. That is, a person will only have one sexual partner at a time. Once a relationship ends, however, the person moves on to a new relationship where they again only have one sexual partner. Looked at in this way, a person could have four sexual partners in four successive relationships over a two-year period but still be monogamous. This person is more accurately described as serially monogamous. (See chapter 7 of your textbook for a discussion of "friends with benefits" relationships).

The concept of serial monogamy is important to consider with respect to the high rates of STI among young people. For example, it is very common for college/university students to be serially monogamous. Many college/university students assume that they do not need to use condoms because they are in a monogamous relationship and neither partner shows any signs of having an STI (note that most cases of STI infection are asymptomatic so the person does not know that he or she is infected). These couples often use the contraceptive pill to prevent pregnancy but take no precautions against HIV/STI. Psychologically, monogamous college/university students often do not consider that they will very likely have different sexual partners in future monogamous relationships. In the end, by the time they graduate from university, many monogamous students will have had two, three, or more relationships, all of them monogamous, none of them protected from HIV/STI. The occurrence of unprotected serial monogamy explains a large proportion of STI infections among college/university students (For discussion of serial monogamy and its associated STI risk among college students see Bolton, McKay & Schieder, 2010).

Key Point

Western culture provides youth with little education or guidance in developing the ability to have sexual relationships characterized by intimacy, mutuality, and pleasure.

3) The sexuality education provided to youth in Western culture, whether it is delivered by parents, schools, or other sources, is often limited to information on preventing sexual health problems such as HIV/STD or is aimed at discouraging youth from becoming sexually active. As we have seen, most people will have their first sexual experiences during the adolescent years but many will do so lacking in accurate knowledge of human sexual response or the skills to have mutually pleasurable sexual relationships. As a result, these first sexual experiences can be a disappointment both physically and emotionally. Adolescents are generally left to trial and error as well as questionable advice from peers or distorted media representations for information on sexual intimacy and pleasure.

Influences on Adolescent Sexuality

Modern communication technologies (e.g., cell phones/smartphones, social media apps/websites) have fundamentally changed the way people are exposed to, learn about, and communicate with respect to sexuality. These technologies can have benefits (e.g., increased access to sexuality information, connecting with relationship partners), but also pose significant challenges (e.g., misinformation, privacy, and exploitation risks of online communication).

Many people will be exposed to media portrayals of gender and sexuality (e.g., via social media, movies/television/music videos/online games, pornography) that challenge sexual health and well-being in either positive or negative ways. Sexual health education programs should facilitate the development of media and digital literacy skills that will enable people to critically evaluate the sexuality-related material they encounter, as well as develop the knowledge and skills to use communication technologies safely and respectfully (SIECCAN, 2019, p. 27).

The examination of adolescent sexuality provides an excellent example of how social context shapes our sexual beliefs, attitudes, and behaviours. As noted above, adolescents living in Western culture receive contradictory messages about sexuality. For example, many parents and religious institutions send the message that teens should not be sexually active while peers and the media may send the opposite message. The sexual health messages contained in school-based programs may be contradicted by the sexually explicit imagery easily found on the internet. Thus, it's not surprising that many adolescents are confused and anxious about sexuality.

Key Point

Adolescents living in Western culture receive contradictory messages about sexuality.

Increasing attention is being paid to the role that the media, particularly television and music videos, plays in influencing adolescent sexuality. Generally, it is assumed that the media's influence is negative. Consider also that adolescents today have much easier access to sexually explicit media (e.g., x-rated videos or pictures) through the internet than did adolescents in the past. For example, a survey of youth in Canada indicated that 22% in grades 7 to 11 reported looking for pornography online and 32% said that they had unintentionally come across pornography without looking for it (Media Smarts, 2022). <https://mediasmarts.ca/sites/default/files/2023-01/Encountering%20Harmful%20and%20Discomforting%20Content%20Report%20-%20YCW%20Phase%20IV.pdf>

Our culture's ambivalence about the reality of adolescent sexuality, combined with the array of contradictory messages about sexuality that adolescents are exposed to, underscores the need to provide effective sexual health education to youth. In Module 9 we will turn our attention to societal attitudes towards sexuality education and the theory and practice of effective adolescent sexual health education.

References

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- McKay, A. & Holowaty, P. (1998). Sexual health education: a survey of adolescents' opinions, self-perceived needs, and current and preferred sources of information. *The Canadian Journal of Human Sexuality*, 6, 29-38.
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- SIECCAN. (2020). *Questions & Answers: Sexual Health Education in Schools and Other Settings*. Toronto, ON: Sex Information & Education Council of Canada. (SIECCAN). https://www.sieccan.org/_files/ugd/1332d5_6506a2c46aba4bb2927143fda80caade.pdf
- SIECCAN. (2019). *Canadian Guidelines for Sexual Health Education*. Toronto, ON: Sex Information and Education Council of Canada. https://www.sieccan.org/_files/ugd/1332d5_e3ee36e39d944009956af5b86f0a5ed6.pdf

Readings

Reading

Required Reading

Human Sexuality in a World of Diversity, 7th Canadian Edition, Chapter (Read only LO12.2: Adolescent Sexuality) and Chapter 15.

Reading for Further Research (Essay Assignments)

Maticka-Tyndale, E. (2008). Sexuality and sexual health of Canadian adolescents: yesterday, today and tomorrow. *The Canadian Journal of Human Sexuality*, 17, (3), 85-95. (Available online - Toronto Metropolitan University library system).

Rotermann, M. & McKay, A (2020). Sexual behaviours, condom use and other contraceptive methods among 15- to 24-year-olds in Canada, *Health Reports*, 31, (9), 3-11. <https://www150.statcan.gc.ca/n1/en/pub/82-003-x/2020009/article/00001-eng.pdf?st=1fEXpUbq>

SIECCAN. (2020). *Questions & Answers: Sexual Health Education in Schools and Other Settings*. Toronto, ON: Sex Information & Education Council of Canada. (SIECCAN). https://www.sieccan.org/_files/ugd/1332d5_6506a2c46aba4bb2927143fda80caade.pdf

SIECCAN. (2019). *Canadian Guidelines for Sexual Health Education*. Toronto, ON: Sex Information and Education Council of Canada. https://www.sieccan.org/_files/ugd/1332d5_e3ee36e39d944009956af5b86f0a5ed6.pdf

Part 2: Learning Objectives

By the end of this part of the module students will be able to:

1. Classify and describe different ideological perspectives towards sexuality education in the schools.
2. Identify and describe Canadian's attitudes towards sexuality education in the schools.
3. Assess adolescent's needs with respect to sexuality education.
4. Identify the key ingredients of effective adolescent sexuality education.

Topic Overview

Introduction

As we saw in Module # 1, and throughout the course so far, the social context of human sexuality in Western culture can be examined in terms of the struggle between the Restrictive and Permissive sexual ideologies. Although Western society appears, over time, to be tilting towards Permissive sexual ideology, there are many sexuality issues upon which Canadian society remains divided. The issue of sexuality education in the schools is also a good case study for examining societal attitudes towards sexuality.

"Sex education is potentially a vehicle for social engineering par excellence, be it progressive or traditional"(Thompson, 1994, p. 40).

"...educators in the field of sexuality inevitably do their work under some assumptions about its purpose and value - assumptions that include a particular vision of the place of sexuality in social behaviour in general and the needs of teenagers in particular" (Zabin & Howard, 1993, p. 113).

In Module # 4, we became familiar with the concept of socialization (The ways in which society conveys to the individual its expectations for his or her behaviour). To a great extent, our sexuality related attitudes and behaviours are the result of the socialization process that occurs as we are growing up. Along with families, friends, religious institutions, and the media, schools are among the most powerful socialization agents in our society. It's not surprising then, that opinions about what should be taught in school-based sexuality education will be a reflection of different ideological perspectives towards sexuality generally.

Key Point

Schools are one of the primary mechanisms for the socialization of youth in our society and, as a result, sexuality education in the schools is often perceived as a powerful force in shaping the sexuality attitudes and values of youth.

Ideology and Sexuality Education: Different Perspectives

Restrictive Sexual Ideology: During the 1970's and 1980's, proponents of Restrictive sexual ideology argued that sexuality education did not belong in the schools. In this view, education about sexuality should be left exclusively to parents and religious institutions. From this perspective, sexuality education in the schools would provoke increased sexual curiosity and behaviour among youth. As it became clear, particularly in the 1990's, that schools would play some role in educating youth about sexuality, proponents of Restrictive sexual ideology tended to modify their position to argue that school-based programs should be focused on the primary objective of teaching youth that sexual activity outside of marriage is immoral, psychologically harmful, and unhealthy. These so-called "abstinence-only" programs have been far more popular in the United States than in Canada. Indeed, the U.S. government actively funds and encourages abstinence programs. Legislation signed into law by U.S. President Bill Clinton in 1996 included a provision that in order to receive federal funding, sexuality education programs must teach "...that a mutually monogamous relationship in the context of marriage is the expected standard of human sexual activity" and "...that sexual activity outside the context of marriage is likely to have harmful psychological and physical effects" (cited in Daley, 1997, p. 3).

Permissive Sexual Ideology: During the 1970's and 1980's, proponents of Permissive sexual ideology argued that sexuality education did belong in the schools. From this perspective, sexuality education in the schools would help to reduce sexuality related social problems such as teen pregnancy and HIV/STI. In this view, knowledge was preferable to ignorance and youth should be taught that sexuality is a natural and positive part of life. Students would be encouraged to clarify their own values about the appropriateness of premarital sex and they would also be informed about birth control methods and ways to reduce the risks of sexual activity (e.g., condom use). Although many school-based sexuality education programs in Canada today make a point of stressing the advantages of abstinence, most programs, particularly in non-religious public schools, do acknowledge the importance of dual protection against pregnancy and HIV/STD for sexually active youth.

A Democratic Approach to Sexuality Education: Is it possible for sexuality education in the schools to avoid indoctrinating youth with a particular sexual ideology? In one respect, it seems clear that as Canadian society has, in some respects, adopted a more Permissive sexual ideology, these shifts in social attitudes have been reflected in our approaches to teaching youth about sexuality. For example, teaching that using condoms is a wise choice for sexually active youth or including the sexual health education needs of LGBTQI2S+ students in school curricula can be seen as examples of sexuality education being informed by Permissive sexual ideology. On the other hand, many sexuality educators and policy makers, particularly in Canada, have suggested a philosophical approach to sexuality education that is based, first and foremost, not in either Restrictive or Permissive sexual ideology, but rather on the basic principles of a democratic society. As noted above, the U.S. government unambiguously sides with Restrictive sexual ideology by funding and promoting abstinence-only sex education. Although in Canada, education is a provincial responsibility, the Canadian federal government has played a role in the past in sexuality education. A notable example of this was the Public Health Agency of Canada's (2008) *Canadian Guidelines for Sexual Health Education* which took a quite different approach than the stance taken by the U.S. government. Consider the following statements from those Canadian guidelines. The Guidelines stated that effective sexual health education:

"is structured so that changes in behaviour and confidence is developed as a result of nonjudgemental and informed decision-making" (p. 11).

"should be provided in an age-appropriate, culturally sensitive manner that is respectful of individual sexual diversity, abilities and choices" (p. 11).

In sum, it would appear that, in contrast to U.S. policy, Canadian approaches to sexuality education reflect basic democratic principles such as the right to self-determination and the right to receive information that enables people to make their own choices.

For a more modern version of these principles see the Core Principles of Comprehensive Sexual Health Education from the 2019 edition of the Canadian Guidelines for Sexual Health Education: www.sieccan.org

Before examining Canadian's attitudes towards sexuality education in schools, let's take a very quick look at the history of school-based sexuality education in Canada and briefly evaluate the quality of current sex education programs.

A Brief History of School-based Sexuality Education in Canada

Up until the 1970's, there were few debates about sexuality education in the schools, largely because few schools had sexuality education programs. Students sometimes received limited information about reproduction and sexually transmitted diseases through lectures on "personal hygiene", or through biology and physical education classes.

Beginning in the 1970's, concern about teenage pregnancies combined with increasing societal openness about sexuality led to the gradual implementation of sexuality education programs in Canadian schools. In the 1980's and 1990's, alarm over HIV/AIDS, high rates of other sexually transmitted infections (STI) in youth, as well as a growing recognition that most Canadian youth were becoming sexually active in their teens, prompted most secondary schools in Canada to introduce some form of sexuality education. By the mid-1990's, every province and territory in Canada either mandated or strongly recommended, through provincial departments of education or health, that some form of sexuality education be taught in the schools (Barrett, 1994). More recently, the province of Quebec has removed sexual health education from its formal curriculum although the Quebec Ministry of Education recommends that teachers find ways to integrate sexual health issues into classroom teaching.

Key Point

Every province and territory in Canada either mandates or strongly recommends that some form of sexuality education be taught in the schools.

The Current Status of Sexuality Education in Canadian Schools

What can we say about the current status of sexuality education in Canadian schools? Although it would be accurate to say that most Canadian youth receive some form of sexuality education before they leave high school, it is also true that few Canadian youth receive sexuality education that provides them with the information and skills necessary to protect and enhance their sexual health.

Key Point

Although most Canadian youth receive some form of sexuality education before leaving high school, few of these programs provide the necessary information, motivation, and behavioural skills that youth need to protect and enhance their sexual health.

The quality of sexuality education in Canadian schools varies from school to school, from school board to school board and from province to province. Thus, it is difficult to generalize about the quality of sexuality education in Canadian schools. However, a number of observations can be made that suggest that there are many weaknesses in contemporary sexuality education.

1. According to data from the Canadian Community Health Survey (Rotermann, 2012; SIECCAN, 2020), about 9% of youth have intercourse before age 15 and another 26% have intercourse at 15 or 16. Yet, many, if not most Canadian youth do not receive detailed education on birth control and condom use until the later grades of high school. In other words, by the time they receive sexuality education, many youth have already become sexually active.
2. Sexual health education programs for adolescents tend to focus primarily on the prevention of sexual problems such as STIs but pay little attention to the enhancement of sexual health and well-being (e.g., healthy, mutually satisfying and pleasurable relationships).
3. Many sexuality education programs in Canadian schools focus on providing basic factual information on the biological aspects of human sexuality. As we will see below, simply providing youth with information on the biological aspects of human sexuality is unlikely, by itself, to result in youth adopting behavioural practices to protect and enhance their sexual health.
4. Very few school teachers have been given specific training on how to provide effective sexuality education programs.
5. Many school principals and administrators believe that providing sexuality education in the schools is highly controversial. They are, therefore, often reluctant to have topics such as sexual orientation and condom use included in the curriculum out of fear that many parents will object to these topics being taught. Catholic schools, in particular, are often reluctant to teach these topics.

Canadian Attitudes Towards Sexuality Education

As noted above, there is a common perception that sexuality education in the schools is a controversial topic and that many parents strongly object to the idea that their children will be taught about topics such as sexual orientation and condom use. To what extent is this true? Some parents do object to these topics being taught because they do not want the schools to contradict the religious or ethnocultural values related to sexuality that they are trying to pass on to their children. However, it is also true that most parents want their children to receive sexuality education in the schools.

Key Point

A strong majority of parents approve of sexuality education in the schools.

Not only do most Canadian parents want sexuality education taught in the schools, parents also want these programs to teach a broad range of topics. Surveys of parents in Canada have found that most parents approve of the teaching of a wide range of topics including healthy relationships, sexual pleasure, masturbation, methods of STI prevention, birth control, gender identity, and sexual orientation (McKay et al., 2014; SIECCAN, 2020; Wood et al., 2021). Most Canadian parents appear to recognize the importance of providing youth with broadly-based sexuality education that addresses the realities of adolescent sexuality and sexual behaviour.

Not surprisingly, surveys of Canadian youth clearly show that they want relevant sexual health education provided in their schools (SIECCAN, 2020). However, these same studies also show that few students are satisfied with the sexual health education they are currently receiving (SIECCAN, 2020). Here is a recent assessment from youth in Canada on their sexual health education experiences and needs: https://www.sieccan.org/_files/ugd/283cae_126a26dfc3db4f09b13a70ddba8c3025.pdf

Working Towards Effective Sexuality Education in the Schools

In your required reading for this module you will learn about the characteristics of effective sexuality education programs. Among the most important characteristics of effective programs is that they use theoretical models of behaviour change to create and teach the program.

Key Point

The *Canadian Guidelines for Sexual Health Education* demonstrate the Information, Motivation, Behavioural Skills (IMB) theoretical model and we will use the IMB model here to illustrate the principles of effective sexuality education. For more information on the key ingredients of effective school-based sexual health education programs see the SIECCAN (2020) document *Questions & Answers: Sexual Health Education in Schools and Other Settings*. (Both of these documents are available online at the [SIECCAN website](https://www.sieccan.org/_files/ugd/1332d5_6506a2c46aba4bb2927143fda80caade.pdf) https://www.sieccan.org/_files/ugd/1332d5_6506a2c46aba4bb2927143fda80caade.pdf https://www.sieccan.org/_files/ugd/1332d5_e3ee36e39d944f

Traditionally, and in many cases today, sexuality education programs focus on providing students with factual information. For example, students might learn that condoms reduce the risk of HIV/STI infection or that not having intercourse means that you don't have to worry about pregnancy. But does simply learning this kind of factual information translate into healthy behaviour? Research has consistently shown that learning factual information is not enough. Students must also be motivated to use the information they have learned and they must also learn the actual skills to incorporate the information into their behaviour. Let's look at two examples (obtaining/discussing partner consent to sexual activity, using condoms, dental dams, or other barriers to prevent STI transmission) of how the IMB model can be used to formulate effective sexuality education for adolescents. Both of these examples are excerpted from the *Canadian Guidelines for Sexual Health Education* (SIECCAN, 2019).

Obtaining/discussing partner consent to sexual activity

Information

Acquire knowledge relating to:

General and legal definitions of clear, affirmative consent and sexual assault

Factors that contribute to non-consensual sexual activity

Understanding nonverbal communication

Discussion of consent within relationships (e.g., consent should not be assumed just because people are already in a relationship)

Communication skills needed to establish shared interest and mutual understanding of desired sexual behaviours

Motivation

Discuss/Address:

Personal attitudes towards the importance of clear, affirmative consent

Ideas/beliefs about personal responsibility to ensure the safety of others

Ideas about perceived social support for obtaining clear, affirmative consent

Perceived costs and benefits of clear, affirmative consent

Personal comfort and competency in navigating conversations related to sexual behaviours, interests, and boundaries.

Behavioural skills

Practicing and applying the knowledge on:

Bringing up and discussing/communicating own consent and obtaining consent from partners (e.g., personal script for obtaining/giving consent)

Assessing verbal and nonverbal consent skills throughout the sexual encounter to ensure there is unambiguous, ongoing, clear, affirmative consent

Bringing up and discussing shared interest and mutual understanding of desired sexual behaviours

Condom Use, Dental Dam Use, or other Barrier Use to Prevent STI Transmission

Information

Acquire knowledge relating to:

Basic information regarding anatomy, physiology, STIs, and pregnancy

STI risk behaviours and transmission

Effectiveness of condoms, dental dams, and other barriers in preventing STI transmission or acquisition

What a condom is, what a dental dam is, what other types of barriers are (e.g., gloves)

Where to access condoms, dental dams, and other barriers (including free and low-cost options)

How to use a condom, dental dam, and other barriers, correctly (e.g., checking expiration dates, applying and removing barriers, storage of condoms, dental dams, and other barriers)

How to discuss condom, dental dam, or other type of barrier use with a partner

Keeping condoms, dental dams, and other barriers available

Comfortable and pleasurable condom, dental dam, or other type of barrier use

Motivation**Discuss/Address:**

Target audience's vulnerability to STIs

Ideas/beliefs about personal responsibility/safety

Ideas about sexual scripts and gender norms that impact condom, dental dam, and other type of barrier use

Individual/peer/public attitudes/norms towards using or not using condoms, dental dams, or other barriers

The specific benefits of condom, dental dam, or other type of barrier use to the individual

Behavioural skills**Practicing and applying the knowledge on:**

How to access condoms, dental dams, and other types of barriers

How to discuss condom, dental dam, and other types of barrier use with a partner

Scripting unilateral use of a condom, a dental dam, or other type of barrier

How to use a condom, dental dam, or other type of barrier (e.g., opening the package, putting a condom on, taking it off; placing the dental dam over a partner's vulva and/or anus so that it creates a barrier between the mouth and the genitals)

Skills for pleasurable condom, dental dam, and other type of barrier use (e.g., assessing condom fit, placing lube on the inside of the condom or dental dam)

How to discuss and engage in alternative types of sex if no condom, dental dam, or other type of barrier is available

These examples illustrate why sexuality education must go beyond providing factual information if it is to meaningfully contribute to adolescent sexual health. These examples also demonstrate that effective sexuality education for adolescents will focus on helping students develop relationship/communication skills. Indeed, surveys of students indicate that they want sexuality education to include topics such as "building equal romantic relationships", "sexual decision-making in dating relationships", "dealing with peer pressure to be sexually active", and "communicating about sex" (SIECCAN, 2019). Unfortunately, however, most school-based sexuality education programs fail to adequately address these topics.

Key Point

In order to effectively contribute to adolescent sexual health, sexuality education programs must include a focus on the development of relationship and communication skills.

In Module # 13 we will look at the *Canadian Guidelines for Sexual Health Education* in more detail and explore how the IMB model can be used to develop, implement, and evaluate sexual health programming for a wide variety of different groups and objectives.

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- Zabin, L. & Howard, S. (1993). *Adolescent Sexual Behavior and Childbearing*. Newbury Park, CA: Sage publications.

Readings

Reading

Required Reading

Human Sexuality in a World of Diversity, 7th Canadian Edition, Chapter 12 (Read only LO 12.2 Adolescent Sexuality) and Chapter 15.

Readings for Further Research (Essay Assignments)

SIECCAN. (2020). *Questions & Answers: Sexual Health Education in Schools and Other Settings*. Toronto, ON: Sex Information & Education Council of Canada. (SIECCAN). https://www.sieccan.org/_files/ugd/1332d5_6506a2c46aba4bb2927143fda80caade.pdf

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